TRIGGERS AND FIXES

Everyday habits fuelling obesity in kids and simple solutions to fight back

Trigger: Night bottles stretching into toddler years
Fix: Stop bottle-feeding by 18–24 months unless it's medically advised

Trigger: Children 'grazing' all day with no meal rhythm
Fix: Offer structured meals—three mains along with two snack windows

Trigger: Sugary milk and juices replacing real food
Fix: Swap liquid sugars for fruit and plain yoghurt

Trigger: Screens invading bedrooms and sleep hours
Fix: Keep bedrooms screen-free to restore healthy sleep

Trigger: Little movement, too much sitting
Fix: Encourage daily physical activity—even in small spaces

After birth, feeding habits deepen these imprints. Rapid weight gain in the first two years—especially among babies fed high-protein formula and large milk volumes—raises later obesity risk. "Overfeeding in infancy is common," says Dr Gupta. "Many mothers breastfeed for extended periods when 20 minutes per breast is sufficient. Prolonged or continuous feeding can contribute to regurgitation, disturbed sleep and excess calorie intake." Infants, thus, should be encouraged to self-feed and develop internal hunger, or satiety regulation, she adds.

THE CULTURE OF EXCESS

Paediatricians say these patterns often stem from cultural norms and anxiety among parents. "For decades, advertising and baby food marketing in India have idealised the image of a round, chubby infant as the picture of health," says Dr Raman Goel, Diabetes and Bariatric Surgeon at Wockhardt Hospital, Mumbai. "That image has stayed in cultural memory. Yet excess weight in infancy is not a sign of health but an early indicator of metabolic stress." The 'Children in India 2025' report, released by the Union ministry of statistics and programme implementation, warns that more than a third of children aged 5-9 show elevated triglycerides and bad cholesterol levels—early markers of heart disease—linked to excess weight.

Modern lifestyles only amplify the risks. Late dinners, sugary snacks and excessive screen time suppress physical activity and distort sleep cycles—each a critical regulator of metabolism. "Sleep is the missing pillar in most obesity conversations," says Dr Sandeep Aggarwal, chairman of the Manipal Institute of Minimal Access, Bariatric, GI & Robotic Surgery, Delhi. "Infants who do not get adequate or good-quality sleep show higher hunger hormone levels and tend to gain more weight. Unfortunately, irregular sleep and late bedtimes are now common even among toddlers."



DR SANDEEP AGGARWAL
Chairman, Manipal Institute of Minimal Access, Bariatric, GI & Robotic Surgery

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Two toddlers illustrate this problem. In Pune, Anaya Singh—born weighing 4.2 kg—was fed sweetened cereals and watched hours of cartoons while her parents worked from home. By age three, she weighed 18 kg and showed early fatty liver changes. In Noida, two-and-a-half-year-old Taran, doted on as *mota bachcha* (chubby baby) by grandparents, was given milk bottles every two hours and cream biscuits through the day. His screen time far exceeded his play time. When his preschool teachers noticed constant thirst and poor attention, blood tests revealed elevated liver enzymes.

RESETTING THE ROUTINE

Most parents, according to Dr Goel, don't even consider that a baby can be overweight, leading to under-reporting. "Infants cannot be blamed for what they eat," he says. "Every calorie comes from an adult who feeds them. In adults, we can talk about willpower or lifestyle, but in babies, obesity reflects the habits of the household."

Newer anti-obesity drugs such as GLP-1 agonists are being tested globally for older children, but those aged under five, experts agree, remain best treated through environment and routine (see *Triggers and Fixes*). "There is no definitive medical treatment for obesity in most young children," says Dr Aggarwal. "In extreme cases, surgery becomes a compassionate, life-saving measure...but these are extraordinary circumstances. What works is sustained behavioural intervention—structured routines, mindful feeding, adequate sleep and physical play. Changing habits at the family level is far more effective than any therapy."

Evidence backs this up. A 2024 review of Indian prevention strategies in the medical journal *Cureus* found that a multipronged approach combining nutrition counselling, active play and screen-time reduction was feasible and effective in preschool settings. Policy can help—by curbing junk food marketing, improving play spaces and educating parents. But the biggest shift must occur at home.

The encouraging fact is that toddlers respond quickly. Their biology is adaptable, their habits still malleable. Early interventions—especially before the school years—can yield lasting results. Ultimately, success won't just show on a weighing scale. It will be visible in a child's energy, confidence and joy in motion—markers of a childhood defined not by excess, but by balance. ■